

# Proposing a Taxonomy of Restriction, Reduction and related Research from a grounded theory involving Needs, Alliance, Shared information, and Help

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## 1 Abstract

Meehl and Golden (1982) defined a taxonomy as the construction of classes and the assignment of things into them. Reducing Restrictive Interventions comes with a penumbra of related ideas. These include Restraint Reduction, restraint, treatment, advocacy, rights-affirmation, risk assessment, initiatives and policies to influence these, and research. The whole constellation may benefit from a taxonomy and the present conference has seen stimulating initial discussion of taxonomies. Classifications exist for restraint: “mechanical”, “cultural”, “chemical”, and for policies, such as the six core strategies. Some other restraint classifications are trivially dichotomous such as long/short or necessary/unnecessary. This submission offers a taxonomy for all of it.

One risk in taxonomy is restraint-creep defined here. Restraint-creep is a social action suffixing bad things or negation of good things with “restraint”. Ad absurdum, perhaps restraint itself is “restraint restraint restraint”. Conversely, borrowing from good practice in computer taxonomies, NASH is “closed” having a fixed overall shape and meaning but “extensible”, able to handle new ideas without creep. NASH taxonomy supposes good care is a co-produced “negotiation” in a “dyad” comprising a person in care and their clinician. The dyad features Needs, Alliance, shared Information, and external Help, having the acronym NASH. NASH sits within the present author’s theoretical framework, adapanomics. A good taxonomy would be exhaustive: all relevant restrictions and harms are breakdowns of that negotiation; all policies influence those; all useful research also. For example classification of restrictions by their rights violated seems fruitful but subsumed by Needs or Help in NASH.

Adapanomics applies the well-established field of microeconomic game theory considering decision making under uncertainty. NASH allows mathematical models, formulation, suggests therapeutic and policy steps, predictions, generalisation, quantification while using every day language. NASH has the strengths of neither focussing on clinicians nor on persons in care.